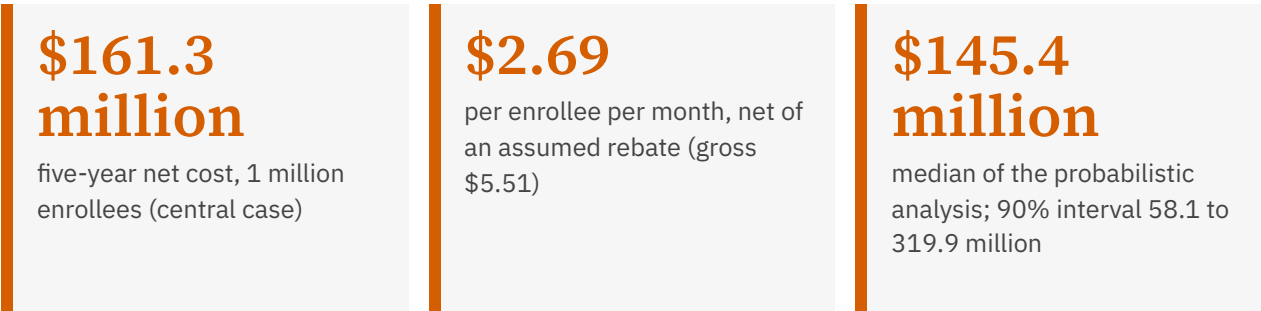


Budget impact of Medicaid coverage of Wegovy and Zepbound

A program of 1 million enrollees, five years, incremental to no coverage

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The central case assumes the prior authorization rules of the covering states (multiplier 1.0), a rebate of 51.2% (midpoint of 23.1% and 79.3%) and a plateau in years 3 to 5. These are scenarios, not forecasts, and no medical cost offsets are included. **Try your own scenario in the [interactive budget model](#).**

What is measured, sourced or assumed

INPUT	VALUE	TYPE
Coverage effect by quarter since coverage began (quarters 1 to 9)	1.3 to 17.2 per 1,000 enrollees per quarter (95% CIs in the Evidence tab)	Measured
Gross cost per prescription	\$1,186 (range \$1,164 to \$1,252)	Measured
Rebate share of gross cost, low	23.1%	Sourced
Rebate share of gross cost, high	79.3%	Sourced
Rebate share of gross cost, central	51.2%	Assumed
Announced net price	\$245 per monthly prescription	Sourced
Prior authorization multiplier	1.0 as observed (tight 0.5 to 0.75; loose up to 1.25)	Assumed
Years 3 to 5 path	Plateau (central), continued growth, or decline to half by quarter 20	Assumed
Purchases per user per year	4.3 (2.9 to 12)	Measured
Adult share of Medicaid enrollment	57.9%	Measured
Eligible share of Medicaid adults	52.0% (lower bound)	Measured
Uptake multiplier	1.0 (0.75 to 1.25 in the sensitivity analysis)	Assumed

Central scenario by year (1 million enrollees)

YEAR	INCREMENTAL PRESCRIPTIONS	MEMBERS TREATED	GROSS COST	NET COST	NET PMPM
1	19,731	4,546	\$23,405,578	\$11,416,541	\$0.95
2	52,407	12,075	\$62,165,691	\$30,322,566	\$2.53
3	68,890	15,873	\$81,717,510	\$39,859,359	\$3.32
4	68,890	15,873	\$81,717,510	\$39,859,359	\$3.32
5	68,890	15,873	\$81,717,510	\$39,859,359	\$3.32

Two per-member costs answer different questions: per user (4.3 fills a year, MEPS) and per member-year of continuous treatment (12 fills).

YEAR	ELIGIBLE POOL	NET COST PER USER PER YEAR	NET COST PER MEMBER-YEAR, CONTINUOUS
1	300,948	\$2,511	\$6,943
2	300,948	\$2,511	\$6,943
3	300,948	\$2,511	\$6,943
4	300,948	\$2,511	\$6,943
5	300,948	\$2,511	\$6,943

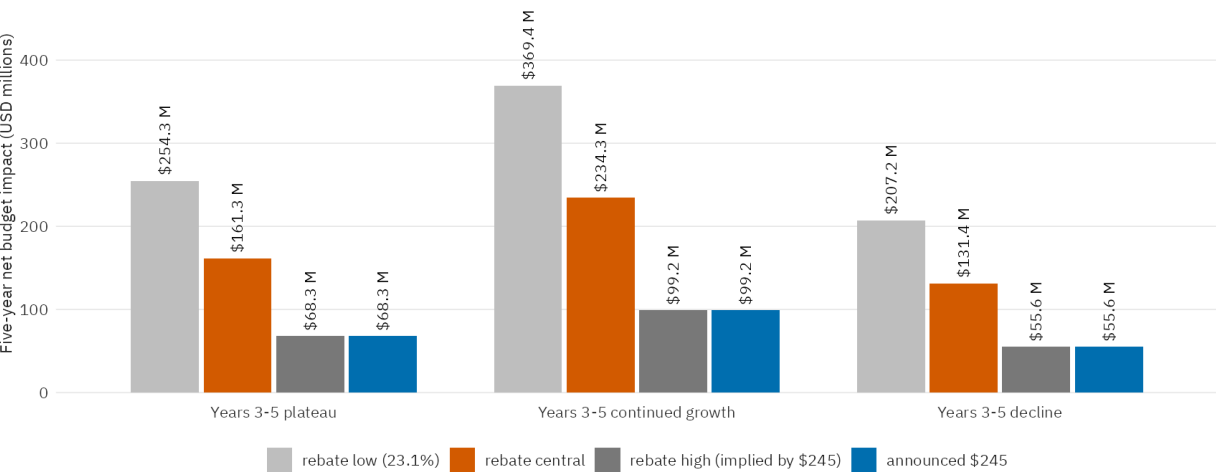
Five-year results by years 3 to 5 scenario and price or rebate

YEARS 3 TO 5	PRICE OR REBATE	GROSS, \$M	NET, \$M	NET PMPM
plateau	rebate low (23.1%)	330.7	254.3	\$4.24
growth	rebate low (23.1%)	480.4	369.4	\$6.16
decline	rebate low (23.1%)	269.4	207.2	\$3.45
plateau	rebate central	330.7	161.3	\$2.69
growth	rebate central	480.4	234.3	\$3.91
decline	rebate central	269.4	131.4	\$2.19
plateau	rebate high (implied by \$245)	330.7	68.3	\$1.14
growth	rebate high (implied by \$245)	480.4	99.2	\$1.65
decline	rebate high (implied by \$245)	269.4	55.6	\$0.93
plateau	announced \$245	330.7	68.3	\$1.14
growth	announced \$245	480.4	99.2	\$1.65
decline	announced \$245	269.4	55.6	\$0.93

Scenarios and sensitivity

Five-year net cost for 1 million enrollees ranges from \$55.6M to \$369.4M across years 3-5 scenarios and rebate or price assumptions; central \$161.3M

Net cost by years 3-5 scenario and by rebate or price assumption (prior authorization as observed, multiplier 1.0; incremental to no coverage).

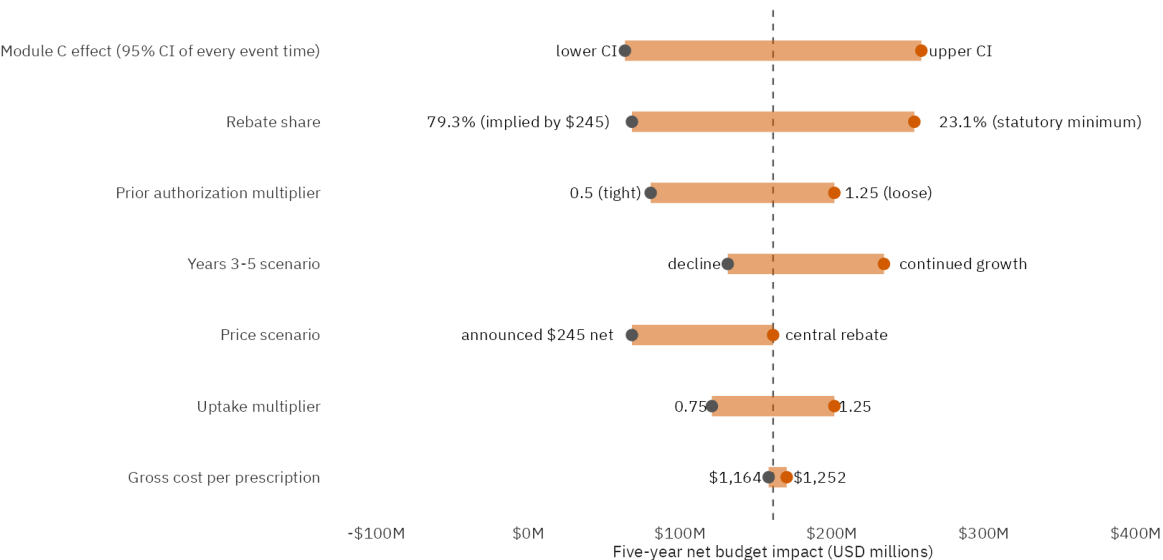


Source: Module C estimates (CMS State Drug Utilization Data, Medicaid enrollment); SDUD gross reimbursement; statutory rebate floor 42 U.S.C. 1396r-8; White House fact sheet Nov 2025.

SDUD amounts are gross of rebates; rebates, prior authorization and years 3-5 are assumptions.

Five-year net budget impact for 1 million enrollees is \$161.3M in the central case

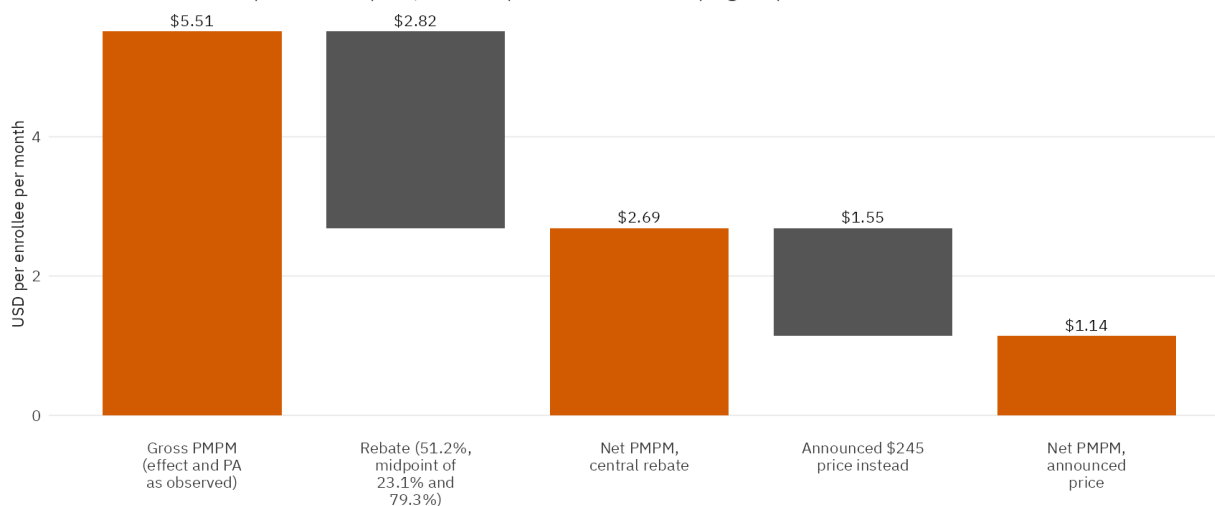
The widest swings are Module C effect (95% CI of every event time) (\$63.7M to \$258.9M) and Rebate share.



Source: Module C estimates (CMS State Drug Utilization Data, Medicaid enrollment); SDUD gross reimbursement; statutory rebate floor 42 U.S.C. 1396r-8; White House fact sheet Nov 2025.

One-way sensitivity: each parameter moved across its range with the others at central values (incremental to no coverage; central = plateau, PA as observed, rebate 51.2% = midpoint of 23.1% and 79.3%). SDUD amounts are gross of rebates; rebates, prior authorization and years 3-5 are assumptions.

Per enrollee per month, a gross cost of \$5.51 falls to a net \$2.69 after a rebate of 51.2% (midpoint assumption), and to \$1.14 at the announced \$245 price (five-year average 1 million enrollees). Waterfall from gross PMPM (effect and prior authorization as observed in the covering states) to net PMPM. The rebate step is an assumption; the \$245 price is an announced program price.

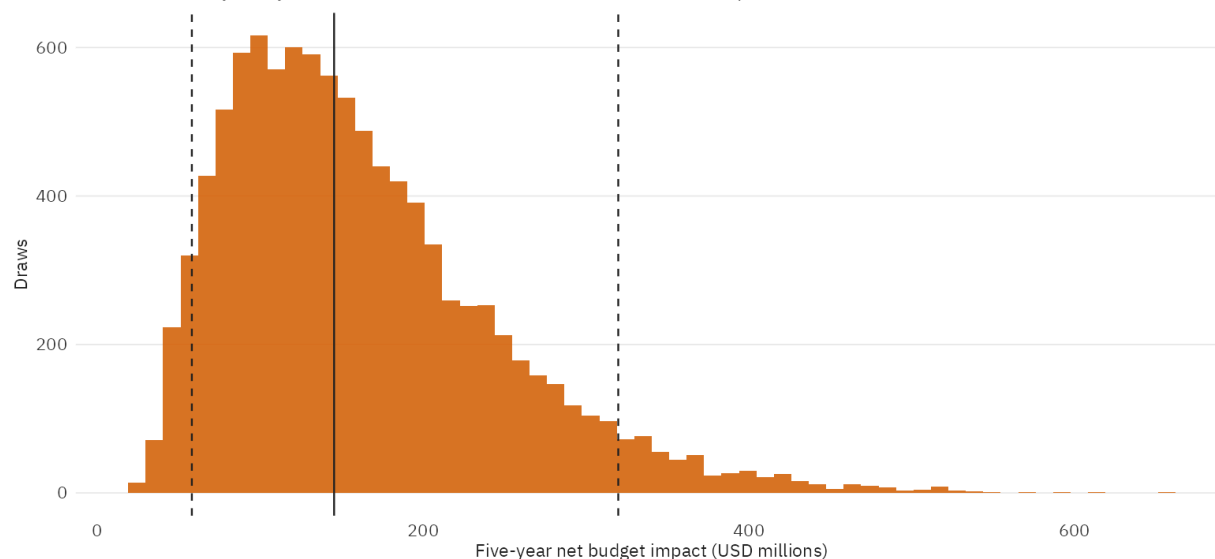


Source: Module C estimates (CMS State Drug Utilization Data, Medicaid enrollment); SDUD gross reimbursement; statutory rebate floor 42 U.S.C. 1396r-8; White House fact sheet Nov 2025.

SDUD amounts are gross of rebates; rebates, prior authorization and years 3-5 are assumptions.

Across 10,000 draws the five-year net budget impact has a median of \$145.4M (90% interval \$58.1M to \$319.9M); PMPM median \$2.42

Probabilistic sensitivity analysis: Module C effects drawn from their SEs (independent event time)



Source: Module C estimates (CMS State Drug Utilization Data, Medicaid enrollment); SDUD gross reimbursement; statutory rebate floor 42 U.S.C. 1396r-8; White House fact sheet Nov 2025.

Perfect correlation gives \$50.3M to \$347.3M), gross cost, rebate, prior authorization multiplier and years 3-5 scenario drawn from their ranges. 1 million enrollees. SDUD amounts are gross of rebates; rebates, prior authorization and years 3-5 are assumptions.

Public aggregate data (CMS State Drug Utilization Data and Medicaid enrollment, NHANES, MEPS); no company affiliation or endorsement. SDUD amounts are gross of rebates. I used AI tools to help write code and documentation; the study design, methods and conclusions are my own, and I verified all results.